

INTRODUCTION

[1] The plaintiff, Mr. Dimopoulos brought an action for damages arising out of a motor vehicle accident that occurred over twelve years ago. The trial took place on May 11, 12, 13, 14, 15, 19, 21, 22, 25, and 26, 2015.

[2] Following my charge to the jury and while the jury was deliberating the defendant brought a motion for a declaration that Mr. Dimopoulos' claim for non-pecuniary damages and chiropractic health care is barred by his failure to establish that his injuries fall within the exception to s.267.5(5)(b) of the *Insurance Act*, R.S.O. 1990.

[3] Commonly referred to as a "threshold motion", for the court to make the requested declaration, it must determine whether the plaintiff has sustained a "permanent serious impairment of an important physical, mental or psychological function". This engages three questions: is the injury the plaintiff sustained permanent, is it important, and is it serious, see *Meyer v. Bright* (1993), 15 O.R. (3d) 129 (C.A.), *Adams v. Taylor* (2013), 118 O.R. (3d) 389, and *Malfara v. Vukojevic* 2015 ONSC 78.

[4] The determination of each of these components is governed by the requirements outlined in sections 4.1, 4.2 and 4.3 of O.Reg. 461/96, as amended by O.Reg. 381/03. These sections clarify the criteria that must be met to show a “permanent, serious impairment of an important physical, mental or psychological function”. Section 4.3 outlines the evidence that a plaintiff must adduce to prove that his or her injuries cross the statutory threshold. In particular, the section requires evidence from one or more physicians and it requires the consideration of very specific questions.

[5] In this case, the jury returned its verdict in the course of the defendant’s submissions on his motion. It awarded general damages of \$37,000 and \$30,000 for chiropractic treatments.

[6] For the reasons that follow, I conclude that Mr. Dimopoulos satisfies the statutory requirements and has suffered a permanent serious impairment of important physical functions. The evidence before the court supports the conclusion that the injuries Mr. Dimopoulos sustained resulted in permanent impairment. The impairment is serious as it includes constant pain that fluctuates in intensity, the inability to sleep through the night because of the pain, and a complete change in Mr. Dimopoulos’ overall disposition from being totally healthy

and 'happy-go-lucky', to being in constant pain, being argumentative and withdrawing from social and family activities. These circumstances, in Mr. Dimopoulos' case makes the impairments important. The defendant's motion is therefore dismissed.

[7] I now turn to review the evidence before the court and my analysis.

THE EVIDENCE

[8] On the subject of the plaintiff's injuries, the Court heard from the plaintiff, his spouse, and his chiropractor. The Court also heard medical evidence from Drs. Cameron and Wilson and Mr. Yip.

a) Evidence of Lay Witnesses

i) Mr. Dimopoulos

[9] Mr. Dimopoulos was forty-nine years old at the time of the accident. He testified that before the accident he was in perfect health and did not even have a family doctor. He said that he did not even know what a headache felt like and could not understand his wife's complaints. He had no sports injuries, no injuries to the neck, the back, or the shoulders. Nor did he have any sleeping problems.

[10] Also prior to the accident, Mr. Dimopoulos said that he played some baseball in the summer before the accident, that he would get together with friends to kick a ball around and that he would also play with his grandchildren in the backyard. He said he took out the garbage and moved furniture around as might be required from time to time. Outdoors, he took care of all the work to the house, including planting, snow plowing and lawn mowing. Though not a licensed mechanic he enjoyed working on cars and would help a friend with used car sales and repairs.

[11] Mr. Dimopoulos explained that his employment history was intermittent. He did not complete high school and over the years he undertook general labour jobs that required significant exertion and heavy lifting. At the time of the accident he was unemployed but in the months leading up to the accident he worked for a movie company as a general labourer and as a security person. That job had both sedentary aspects and demanding physical labour. The latter related to the handling and movement of heavy power cable for various movie sets. Mr. Dimopoulos admitted to some irregularities in the reporting of his income in certain years prior to the accident. He attributed those irregularities to his limited income and his unemployed status at that time.

[12] Mr. Dimopoulos said that his injuries consisted of constant pain to the neck, and pain in his shoulders and his lower back. Six months following the accident he remained in pain. He said that since the accident he experiences regular headaches and has serious difficulties with his sleeping. He also said that since the accident he has never been pain-free and that the pain fluctuated in severity. He explained that he has been unable to sleep through the night because of the pain. Initially, he took medications such as Percocet for some time but he had serious concerns about becoming addicted to the medication.

[13] In terms of his activities post-accident, Mr. Dimopoulos testified that he feels useless. He said that he stopped attending family functions and outings or visits with friends because he found it too difficult to stay still for too long. He said he would have to get up and walk around or stretch to relieve the pain he would feel. He considered that disruptive. Rather than draw attention to himself, he preferred not to participate in family and social activities.

[14] Regarding his employment, Mr. Dimopoulos did not attempt to return to work with the movie production company after the accident because he did not believe that he could do any of the physical work associated with it. In 2008, he obtained a job with Utility Energy Services as a Pipeline Inspector. That job does not entail any physical labour. He drives around and patrols pipelines over long

distances. He will get out of his car at least a dozen times to check particular constructions sites and compounds. He said that he did this job but often had difficulties concentrating because of his difficulties with his sleep. He said that although he was able to his job, he did so with constant pain. He also indicated that it was fortuitous that he obtained the particular job; his wife worked there as well and she facilitated an introduction. He worried that if he lost the position he would have a very difficult time finding alternate sedentary employment, given his restrictions.

[15] Mr. Dimopoulos also testified that his relationship with his wife has changed significantly since the accident and that his continuous pain causes him to argue with her. His difficulties with his sleeping makes him irritable and impatient. That only feeds into disagreements. He also explained that his intimate relationship with his wife changed drastically after the accident.

[16] On cross-examination, Mr. Dimopoulos was asked about his activities in and outside of the house. He was also shown several hours of surveillance which showed Mr. Dimopoulos coming in and out of his home, doing some work on a car parked in the driveway, going and spending time at the casino, and doing his driving shifts and inspections. The surveillance did not capture Mr. Dimopoulos

doing any heavy lifting or any demanding chores. His activities were sedentary in nature. The surveillance did show Mr. Dimopoulos turning his head to check blind spots as he pulled out of parking spots.

[17] Mr. Dimopoulos was asked to agree that the surveillance showed him “living a pretty normal life” and that “there doesn’t appear to be too much wrong with you in those videos”. Mr. Dimopoulos disagreed; he focused on how he feels when he undertakes his activities. It was also put to him at trial that there was very little that he was unable to do following the accident. This question was also put to him at his discovery. This is the exchange on this point:

- Q. What I am saying to you is, is that there’s not too much that you’re unable to do. Is that a fair description?
- A. It it’s very light I can possibly do it, but like I said, I have to do it with pain. The pain is always there, it’s not leaving me, that’s the...
- Q. In general ...
- A. ... bottom line.
- Q. ... I’m not talking about light duties, things like that, in general, would you agree with me that there’s very little you’re unable to do?
- A. I don’t know how to ...
- Q. Yes or no, Mr. Dimopoulos?
- A. No
- Q. No, you do not agree with me?
- A. No, I don’t.

Q. Okay. All right, Your Honour, back to the [discovery] transcript. So p.199, question 1098, "What do you say". Question, "What do you say you can't do following the accident that you still can't do today and that you attribute to an injury from the accident?" Answer, "The thing is it's not too much that I can't do, it's the aftermath after I do it. That's when the pain sets in two or three days later or the next night. If I'm shoveling snow, which I have to do, my wife can't get out there, she's four foot nothing, she's got osteoporosis and arthritis and she can't lift a shovel. She can't even move it so I have to do it and then I pay for it after that." Were you asked that question and did you give ...

A. Yes ...

Q. ... that answer?

A. ... and that is the answer I gave.

Mr. Mester: Okay, thank you, no further questions.

[18] On re-examination in response to questions from his counsel, Mr. Dimopoulos said:

Q. ...During cross-examination, counsel put to you that the surveillance videos depict you living an active and normal daily life and you disagreed?

A. Yes.

Q. Why did you disagree?

A. Well because I'm not active. I don't lead an active life because I'm always in pain. I mean I'm in pain all the time. I mean the pain fluctuates, sometimes it's a little stronger than other times, but it is always there. I mean that's not an active normal life. I can't do things I want to do. ...

[19] Mr. Dimopoulos said that he turned to chiropractic treatments for relief to his pain.

ii) Ms. Joyce

[20] Mr. Dimopoulos' spouse, Ms. Joyce, corroborated Mr. Dimopoulos' evidence. She said that prior to the accident Mr. Dimopoulos was "easy going, full of fun, fun loving, always smiling and loveable". Since the accident she said that Mr. Dimopoulos did not enjoy life any more. She said that he was not loveable anymore, he was not easy going and was not fun loving, and he didn't enjoy anything. She then clarified that she did not mean to say that he did not love her and her daughters but that because of his constant pain, "he pulled away" because he could no longer enjoy family activities. She explained that Mr. Dimopoulos gradually began to isolate himself from friends and family because he could not stay still at such visits; he has to get up, fidget, and walk to manage his pain.

[21] In the 12 years since the accident, Ms. Joyce said that things were getting worse for Mr. Dimopoulos. She said that the pain intensity fluctuated but that the pain was constant. She estimated that Mr. Dimopoulos had slept through the night in only six to seven times over the past twelve years and that he had difficulties with physical labour. She acknowledged that he did do some outdoor work but that he would suffer from increased pain for a number of days following the activity.

[22] Ms. Joyce pointed out that Mr. Dimopoulos always had difficulties finding employment because of his limited education. However, where in the past he could pursue jobs that required physical labour, he would no longer be able to pursue such options if he were to lose his current sedentary job.

iii) Dr. Lavis

[23] Dr. Lavis, who has been Mr. Dimopoulos' treating chiropractor testified that he treated Mr. Dimopoulos regularly since 2003 and that in 2013 and 2014 he would have treated Mr. Dimopoulos between two and three times a week. He said that he did not record the vast number of visits in 2013 and 2014 because he would not charge Mr. Dimopoulos. He said that he knew that Mr. Dimopoulos could not pay for the treatment but he also was of the view that Mr. Dimopoulos needed the treatments. He said that his record-keeping practices were not in accordance with the profession's requirements but that was his way of recording visits. He also indicated that his record-keeping approach was not unique to Mr. Dimopoulos; he said that he did this with other patients. He explained that he did this as a result of concerns over the accounting and the accumulation of accounts receivable. He expressed the hope of being compensated for his efforts if Mr. Dimopoulos received compensation but he did not require or expect compensation.

b) Medical Evidence

[24] The Court heard from Dr. Cameron, Dr. Wilson and Mr. Yip.

i) Dr. Wilson

[25] Dr. Wilson gave expert testimony on behalf of Mr. Dimopoulos. He was qualified as an expert in Orthopaedic Spinal Surgery, soft tissue injuries, chronic pain and treatment of orthopaedic surgery and soft tissue injuries and causation as it pertains to a party injured in a motor vehicle accident.

[26] He encountered Mr. Dimopoulos on two occasions in 2007 and in 2010. In 2007, in response to a referral request from Mr. Dimopoulos general physician, Dr. Wilson declined the referral because he did not feel there was anything that he could treat. He recommended conservative care which at trial he defined to include massages, botox injections, chiropractic care, acupuncture and cortisone injections.

[27] Dr. Wilson conducted a full examination and assessment of Mr. Dimopoulos in February 2010. He concluded that as a result of the motor vehicle accident of October 18, 2003, Mr. Dimopoulos could have sustained a soft tissue injury to the cervical spine, commonly referred to as a “whiplash associated

disorder". He described it as a Grade II injury. He also concluded that Mr. Dimopoulos sustained a soft tissue injury to his lower back and his left shoulder, as well as mechanical back pain. He considered the injuries to be permanent. Dr. Wilson concluded that Mr. Dimopoulos could undertake sedentary activities of daily living. However, he concluded that Mr. Dimopoulos could not do the heavier tasks of daily living.

[28] In his testimony, Dr. Wilson explained that Mr. Dimopoulos showed signs of chronic pain throughout his examination. He did not find Mr. Dimopoulos to be exaggerating his symptoms. He said that his assessment included testing to detect duplicity but that he did not detect any such behaviour. He agreed that pain is subjective that there was no machine to test the existence of pain. But he also agreed that there was objective testing to evaluate the existence of the reported injuries.

[29] In terms of a prognosis, Dr. Wilson concluded that the neck and back injuries were permanent and that they could get worse in the future. He also concluded that the chronic pain would shorten Mr. Dimopoulos' occupational career.

ii) Mr. Yip

[30] Mr. Yip was qualified as an expert witness in physical therapy and in administering Functional Abilities Evaluations. He conducted a Functional Abilities Evaluation on Mr. Dimopoulos on June 24, 2013 over two and a half hours. He noted that Mr. Dimopoulos gave a full test effort during the isometric tests for strength. He pointed to objective measures such as Mr. Dimopoulos' muscle recruitment, his breathing pattern during the testing his facial grimacing and his increased heart rate to support his conclusions regarding Mr. Dimopoulos' efforts.

[31] Mr. Yip concluded that Mr. Dimopoulos was not able to perform the essential tasks of his job as a general labourer and did not meet 9/16 demands of the testing because of physical limitations. The particular difficulties he noted related to difficulties lifting, carrying, pushing, standing, stopping, squatting, and climbing. He concluded that Mr. Dimopoulos could function at a "light strength level" and only on an occasional basis.

iii) Dr. Cameron

[32] The court also heard evidence from Dr. Cameron who was qualified as an expert in orthopaedic surgery, soft tissue injury, chronic pain, and the treatment of orthopaedic and soft tissue injury and causation in these areas as it pertains to

the plaintiff. He testified that Mr. Dimopoulos suffered a soft tissue strain of the neck and back. He concluded that there was no objective evidence of any significant organic orthopaedic pathology. He said that soft tissue injuries typically heal within a few weeks or a few months of an injury. Based on his ten minute examination of Mr. Dimopoulos he concluded that there was ample evidence of symptom magnification and he demonstrated to the jury some of the tests he conducts to verify if an individual is misrepresenting his pain. Those tests involved hip rotation and knee rotations.

[33] Dr. Cameron did not believe that there was a causal connection between the shoulder pain and the motor vehicle accident. He also stated that the chiropractic treatment was detrimental to Mr. Dimopoulos.

ANALYSIS

[34] Mr. Dimopoulos submitted to the court that he met the burden of proving on a balance of probabilities all of the requirements of s. 267.5(3) and (5) of the *Insurance Act*, and ss.4.2 and 4.3 of O.Reg. 461/96.

[35] The defence argued that as a starting point, Dr. Wilson's evidence did not satisfy the requirements of section 4.3. On that basis alone he concluded that the

Court could grant the defendant's threshold motion. In addition, he argued that Mr. Dimopoulos' ability to work full time and earn "600%" more than he did prior to his accident, his ability to shovel snow, to cut the grass, to collect leaves and to use a weed whacker for the weeds, combined with his statement that there was very little that he could not do, led to the inevitable conclusion that Mr. Dimopoulos did not sustain a serious impairment. Finally, the defence relied on Dr. Cameron's view that the vast majority of people who sustain soft tissue injuries from a motor vehicle accident heal within a few months of the accident and that in his examination of Mr. Dimopoulos he found that he magnified and misrepresented his symptoms.

a) Findings

[36] In my review of the evidence, I am unable to agree with the defendant's submissions for a number of reasons.

[37] Beginning with the medical evidence, I find that Dr. Wilson's evidence met the requirements of section 4.3(1)-(5) of O. Reg. 381/03 of the *Insurance Act*, R.S.O. 1990, c.1.8 as amended. He was qualified as an expert in soft tissue injuries, chronic pain and the treatment of soft tissue injuries and causation as it pertains to injuries and complaints by a patient injured in a motor vehicle

accident. He based his opinion on medical evidence in accordance with generally accepted guidelines or standards of the practice of medicine.

[38] Dr. Wilson's assessment of Mr. Dimopoulos was based on a full examination of him of over two hours. In his report and his testimony, he discussed the nature of Mr. Dimopoulos' impairment, he confirmed its permanence and he suggest that with age, the pain could get worse. Specifically, Dr. Wilson noted that from his examination of Mr. Dimopoulos, "he had restrictions to movements and mechanical issues" that he felt would not allow him to return to physically challenging activities. He thought that Mr. Dimopoulos could engage in sedentary activities. Dr. Wilson did not think that there was any potential for improvement; he concluded that the injuries were permanent.

[39] In sum Dr. Wilson concluded that the injuries to Mr. Dimopoulos resulted from the accident, that Mr. Dimopoulos would not improve, he would continue to experience the pain symptoms and that would have a negative impact on his daily living, his recreational endeavours and his occupational endeavours. Finally he was firm in his explanation that he did not detect any symptom magnification by Mr. Dimopoulos. He explained how in his examination, he would test for such a possibility.

[40] Qualitatively, Dr. Wilson's expertise and knowledge was extensive and impressive. His evidence was thorough. He spoke in measured terms and was very clear in his explanation of the differences between soft tissue and other injuries, and the meaning of chronic pain. He withstood cross-examination and was very professional. He explained Mr. Dimopoulos' injuries and his limitations in a way that reflected a thorough understanding of Mr. Dimopoulos' condition.

[41] In contrast to Dr. Wilson, Dr. Cameron doubted Mr. Dimopoulos' complaints and rejected the conclusion that he suffered from a serious impairment. He demonstrated the tests and the reasons for which he came to his conclusions. He agreed that his examination of Mr. Dimopoulos was no more than ten minutes but he stood by his conclusions. Dr. Cameron spoke in very absolute terms.

[42] As between the two medical doctors, I found Dr. Wilson to be more thorough and convincing than Dr. Cameron. I was persuaded by his conclusions concerning the permanence and the seriousness of Mr. Dimopoulos' impairment. My preference for Dr. Wilson's evidence over Dr. Cameron is also shaped by their very different approaches to their opinions. There is no question that they were both qualified and that they both presented with extensive experience.

Neither side challenged their independence in any serious way. However, their approaches to Mr. Dimopoulos' condition were revealing.

[43] Dr. Wilson focused in large measure on Mr. Dimopoulos' subjective complaints. Having satisfied himself that Mr. Dimopoulos was not malingering. He discussed the effect of the injury on Mr. Dimopoulos. He acknowledged that most patients would recover from soft tissue injuries such as those sustained by Mr. Dimopoulos but that did not negate his assessment for Mr. Dimopoulos. He noted that there were exceptions.

[44] In contrast, Dr. Cameron's principle point of departure was the labelling of Mr. Dimopoulos' injuries. His analysis was constructed around the view that most people recover from soft tissue injuries and that should have been the case with Mr. Dimopoulos. Although Dr. Cameron demonstrated in a rather colourful and impressionistic manner his reasons for which he did not believe Mr. Dimopoulos, I was left with the distinct sense that the labelling of the injuries informed Dr. Cameron's suspicions. His evident disdain for chiropractors merely reinforced his conclusions.

[45] The distinction between the two approaches is significant because courts have repeatedly recognized in the context of determining the seriousness of an

impairment there is a need to consider “the effect of the injury” on a claimant as opposed to the “type of injury,” see, *Malfara v. Vukojevic*, supra., at para. 23.

[46] Next, I turn to the plaintiff’s corroborative evidence to the evidence of Dr. Wilson. I note that a plaintiff is not excluded from corroborating the evidence of a physician. He is also not required to adduce evidence in addition to his own evidence to corroborate the evidence of the physician, see: *Gyorffy v. Drury*, [2013] O.J. No. 3108.

[47] On the issue of the nature of Mr. Dimopoulos’ injuries and their effects the Court heard from Mr. Dimopoulos as well as his wife, Ms. Joyce and Mr. Yip. Both Ms. Joyce and Mr. Yip were credible witnesses. Their respective evidence corroborated Dr. Wilson’s evidence, as well as that of Mr. Dimopoulos. Both testified that Mr. Dimopoulos could not undertake activities that required significant physical exertion. Like Dr. Wilson, Mr. Yip did not find symptom magnification by Mr. Dimopoulos. His explanation of the objective measures for the detection of duplicity was especially convincing. I find that his description of Mr. Dimopoulos’ limitations dovetailed with Dr. Wilson’s conclusions. Like Dr. Wilson, Mr. Yip concluded that Mr. Dimopoulos could undertake light tasks and sedentary activities.

[48] Ms. Joyce was unshaken in cross-examination. I disagree with the defence's submission that Ms. Joyce's evidence should be rejected as largely incredible because she stood to gain from a determination in Mr. Dimopoulos' favour. I did not detect any attempt to exaggerate, overstate, or magnify her observations. Ms. Joyce's testimony was candid, spontaneous and clear. She brought into focus the dramatic changes to Mr. Dimopoulos before and after the accident. She verified that Mr. Dimopoulos was so healthy before the accident that he did not even have a family doctor. She did not hesitate to speak of Mr. Dimopoulos' weaknesses. Her worry that if Mr. Dimopoulos lost his current sedentary job, he would have a hard time finding another sedentary job was genuine and credible.

[49] I turn to my findings concerning Mr. Dimopoulos' evidence. For starters, the defence's repeated attempts to dilute Mr. Dimopoulos' responses as they related to the activities that he could undertake were troubling. While it is entirely appropriate to test a witness' answer(s) to a question, the attempt to re-phrase the answers, by leaving out material parts, or by cutting off the response to a question undermines the quality of the questioning and by extension weakens the weight that a court may then place on the corresponding answers. This

difficulty was evident in both the discovery questions that preceded the trial and in the questions that were put to Mr. Dimopoulos following the presentation of the surveillance at trial.

[50] With respect to the question put to Mr. Dimopoulos at discovery to the effect that there was nothing that Mr. Dimopoulos could not do, I find that at no time did Mr. Dimopoulos give an unqualified answer in support of what was proposed to him. His full answer included a crucial qualifier. Mr. Dimopoulos, repeatedly and consistently explained that when he had to take on heavy tasks, he then had to endure a significant increase in his pain levels in the following days. Moreover, the instances where he undertook heavy tasks were occasional. At no time in his evidence did he say that he was able to undertake the tasks either his pre-accident task without pain or even with tolerable pain.

[51] In re-examination, Mr. Dimopoulos repeated his qualification concerning the effects of taking on the more demanding physical day to day tasks. The defence suggested to the court that Mr. Dimopoulos' answers in re-examination reflected an evolution and variation in his responses and that the court should reject the response and prefer the response from Mr. Dimopoulos' discoveries.

[52] In my review of Mr. Dimopoulos' responses on the two occasions, I am unable to identify the suggested evolution in his answer. Mr. Dimopoulos was consistent in his evidence on the effects and the fluctuation in his ongoing pain. Although he agreed that when necessary, he undertook demanding activities, he consistently explained that he "paid" for such activity with increased pain. He went on to explain that chiropractic care was one way he obtained some relief. Mr. Dimopoulos would be in a totally different situation if his answer amounted to a bald admission that there wasn't much that he could not do. But that was not the answer he gave at any time.

[53] Regarding the surveillance, I find that the questions that were put to Mr. Dimopoulos following the viewing of the surveillance were unfair. The defence suggested that the surveillance depicted a day of normal daily activities and that Mr. Dimopoulos seemed to be enjoying a normal daily life. Mr. Dimopoulos' initial confusion over this suggestion was understandable and his answer that one could not see the pain was a reasonable response. The surveillance was actually unremarkable and did not depict anything that was inconsistent with Mr. Dimopoulos' evidence. For example, the surveillance did not show Mr. Dimopoulos undertaking any activities that required any particular exertion. He

was not seen doing any shovelling, any raking of the leaves, or any activities that would suggest an ability to exert himself beyond sedentary and light activities.

[54] Having seen the surveillance, I am not sure what the defence meant when it was suggested that it depicted normal daily activities and a normal life. If anything, the surveillance validated Mr. Dimopoulos' complaints. It showed Mr. Dimopoulos walking in a guarded and deliberate manner. His movement at times looked like it was in slow motion. His guarded gait was similar to what was observed in court, suggestive of a distinct discomfort. He did turn his head in left and right directions when he drove and backed out of parking spaces but there was no way to conclude that the neck-turning was pain-free or even unrestricted. The activities depicted reflected a typical day for Mr. Dimopoulos. It is a far cry to suggest that Mr. Dimopoulos' normal activities equate with some generic normal day.

[55] Separate and apart from an attempt by the defence to convert some of Mr. Dimopoulos' answers into admissions of leading a normal life, much of the defence's theory was anchored on the portrayal of Mr. Dimopoulos as somebody who was incredible and whose subjective assessment of his pain was exaggerated.

[56] The issue of credibility is especially important in a case where the source of the pain is soft tissue injury and where the magnitude of the pain cannot be measured objectively. The Supreme Court of Canada has recognized chronic pain but it has also recognized that by definition, the existence of chronic pain is not supported by objective findings and that those suffering from such pain are subjected to persistent suspicions of malingering, see *Martin v. Nova Scotia (Workers' Compensation Board)*, [2003] 2 S.C.R. 504. See also *Mayer v. 1474479 Ontario Inc.*, [2013] O.J. No. 4945, at para. 22..

[57] I find that on balance, Mr. Dimopoulos was credible in his description of his limitations, the pain he suffered, and his management of that pain. He was remarkably consistent in his description of the pain he has been enduring and the limitations over the twelve years. There was some concern that Dr. Bath's notes in and around the time of the accident did not record pain in the shoulder, but there was a reference to localized pain to the upper trapezius. Dr. Wilson spoke of the pain coming from the parascapular musculature, meaning the muscle that attaches to the scapula, which is the shoulder blade. Dr. Cameron drew a distinction between the shoulder, the parascapular muscle and the trapezius but most significantly saw no connection between the alleged shoulder pain and the accident.

[58] On the evidence before me, I am unable to reconcile the missing reference to shoulder pain in Dr. Bath's notes. However, even if that particular pain were discounted, the balance of Mr. Dimopoulos' complaints was consistent. Both Dr. Wilson and Mr. Yip considered those complaints and concluded that they resulted in serious limitations. I consider their evidence a full response to the defence's suspicions and allegations of malingering by Mr. Dimopoulos.

[59] Insofar as the defence relied on past irregularities to Mr. Dimopoulos' income tax claims for one more indicator to support the proposition that Mr. Dimopoulos was a liar, I did not agree with that conclusion. Mr. Dimopoulos was candid about the particular failing. His explanation was in keeping with what amounted to a poor employment record prior to the accident. I also note that the sum at issue was very minor. The implications would be very different if the failure to disclose concerned a significant sum of unreported income.

b) Application of Findings to the Law

[60] In light of these findings, I turn to the three questions to determine whether or not Mr. Dimopoulos suffered a permanent serious impairment of an important physical function.

i) Did Mr. Dimopoulos sustain permanent impairment of a physical function?

[61] Based on my findings regarding Dr. Wilson's evidence and findings, I conclude that Mr. Dimopoulos sustained a permanent serious impairment of a physical function. In his testimony in court, Dr. Wilson noted that Mr. Dimopoulos "had restrictions to movements and mechanical issues", such that he concluded that Mr. Dimopoulos was not going to be able to return to "the more heavy physically challenging activities but that he would continue with the "sedentary type of stuff". He did not see any potential for improvement and therefore concluded that Mr. Dimopoulos' injuries were permanent.

ii) If the answer to the first question is "yes", then is Mr. Dimopoulos' bodily function, which is permanently impaired an important one?

[62] The permanent impairment of Mr. Dimopoulos' bodily functions are important. When considering this question, the court must consider the injured person as a whole and the effect of the impaired bodily functions on a person's life in the broadest sense of the term. If the bodily function is important to the particular injured person, then the bodily function is an important one, see *Meyer v. Bright, supra*.

[63] Mr. Dimopoulos was physically and mentally very healthy prior to the accident, so much so that he did not even have a family doctor. He described himself as active, happy, and he participated in some sports with his friends and supported both his wife and step children. He went from being happy, accommodating, and fun-loving, to be angry, grumpy and argumentative with his wife. His constant pain has also had a significant impact on his sleep. He complained that he has had virtually no nights of uninterrupted sleep since the accident. That lack of sleep has an effect on his moods and his overall stamina. It also had an effect on his intimacy with his wife.

[64] Given his limited education, the loss of his ability to undertake heavy duties and labour activities is significant because that poses the risk of being limited in his ability to find work. It is acknowledged that he was lucky to locate sedentary work in 2008 that actually resulted in earnings that exceeded Mr. Dimopoulos' earnings prior to the accident. But, I also recognize that the particular job was fortuitous, it has some unique aspects to it, and if Mr. Dimopoulos were to lose that job, he would have a difficult time finding other sedentary work for which he would qualify. Given his age, re-training for other sedentary work would be an unlikely option.

[65] Taken together, the inability to sleep through the night, the constant pain to the neck, the back, as well as the shoulder area and its effects on Mr. Dimopoulos overall psychological disposition, and the limitations on the undertaking of physical activities are important activities for Mr. Dimopoulos.

iii) If the answer to the second question is “yes”, is the impairment of the important bodily function serious?

[66] I conclude that the Mr. Dimopoulos’ permanent impairments to important bodily functions are also serious. This term, as reflected in the components contained in section 4.2 of the threshold regulations import a sense of degree. A serious impairment is one that causes substantial interference with the ability of the injured person to perform his or her usual daily activities or to continue his regular employment. An impairment is serious if it substantially interferes with most of the plaintiff’s usual activities. See, *Mayer v. 1474479 Ontario Inc.. supra*, *Bowman v. Lovett*, [2007] O.J.No.462, and *Malfara v. Vukojevic, supra*.

[67] As noted above, the effect of the injury is what is important in the consideration of this question and not the type of injury or the labels that might attach to an injured party’s complaints. “The effects of chronic pain are just as

real and just as likely to meet or not meet the threshold as any other type of injury or impairment. It all depends on the manner in which the plaintiff has been impacted. The threshold determination is to be done on a case by case basis, see *Malfara v. Vukojevic, supra.*, at para.23”.

[68] In the context of this analysis it is also important to note that an injured party must do more than simply experience ongoing pain or discomfort to bring him or herself to meet the threshold requirements. Courts have recognized that in introducing Bill 198 the Legislature intended the injured party to bear some interference with their enjoyment of life, without being able to sue. In that regard, tolerable symptoms that permit a claimant to function well would not satisfy the statutory exceptions. Symptoms that go beyond being tolerable and significantly impair a person’s enjoyment of life will be sufficient. A court must look at the totality of the evidence to assess and determine whether the interference is substantial. “A person who can carry on daily activities, but is subject to permanent symptoms that have a significant effect on her enjoyment of life, (e.g. because of sleep disorder, headaches, dizziness and nausea), must be considered as having sustained a serious impairment, in the sense required.” See, *Mayer v. 1474479 Ontario Inc., supra*, at para.18, quoting *Mayer v. Bright, supra*, at p.19, *May v. Casola*, [1998] O.J. No. 2475 (C.A.), *Hartwick v. Simswer*,

[2004] O.J. No. 4315 (S.C.J.), *Franfurter v. Gibbons*, [2004] O.J. No. 4969 (Div.Ct.), *Pinchera v. Langille*, [2005] O.J. No. 521 (S.C.J.), *aff'd* [2006] O.J. No. 3948 (C.A.), and *Vancsody v. Wrightman*, [2012] O.J. No. 6517 (S.C.J.).

[69] It is also recognized that frustrations to career paths, changes in job functions are considered serious. Activities of daily living are wide-ranging and go beyond household and employment activities to include the ability to socialize with others, have intimate relations and enjoy one's family and recreational activities, see *Mayer, supra, at para.18*.

[70] The impairments sustained by Mr. Dimopoulos have had a detrimental effect on his overall disposition. He is different from the day before the accident. He went from being completely healthy to having constant pain. He went from being social, happy go lucky, to being miserable, argumentative, and feeling, to use his word, "useless". At no time did he say that he was able to resume a normal life without any limitation or, with minimal or tolerable limitations. His daily activities are sedentary in nature. He readily admitted that he will undertake heavy chores from time to time but he also indicated that he paid in the days following such an activity with increased pain.

[71] Although the defence argued that Mr. Dimopoulos was no different than the plaintiff in *Malgara*, who also gave evidence of having to tolerate pain and having to pay for it later, there are also material distinctions in the factual matrix of that case. The most significant difference between the two cases lies with the fact that in *Malgara*, the court found that “the plaintiff resumed his full-time physically demanding employment as a plumber’s apprentice within a few weeks without notable restriction, without ongoing debilitating pain and with no loss of pay. He went on to become a licensed plumber. The injuries sustained have not prevented him from becoming and working as a full-time plumber. There has been no or little restriction in his ability to carry out his duties as a plumber”, see *Malgara*, at para. 29.

[72] This is not Mr. Dimopoulos’ situation. His “normal life” changed after the accident. He has not been pain-free since the accident. He has not been able to sleep through the night, he has become withdrawn from family and social activities and his personal life with his wife has been affected very significantly. In the totality I am satisfied that Mr. Dimopoulos’ injuries have had a significant effect on his enjoyment of life given his age. His impairments have substantially interfered with most of his usual activities of daily living. The totality of the functions that have been impaired are important to Mr. Dimopoulos’ usual

activities of daily living and his pre-accident combined activity capacity for work or pleasurable activity.

c) The Jury's Verdict and its Implication on this Motion

[73] Finally, since the jury returned a verdict in the course of the argument of this motion, both counsel made submissions on the implications of their conclusions. The defence argued that the low awards, relative to what the plaintiff asked of the jury underscored the weaknesses in Mr. Dimopoulos' credibility. The plaintiff argued otherwise. Although, counsel acknowledged that the awards were lower than what was requested, he noted that they were not insignificant.

[74] It is important to observe that our civil jury systems and the superimposed legislative scheme contained in the *Insurance Act* means that the jury, charged with factual determinations and a judge charged with making a legal threshold ruling which effectively requires factual determinations on the same or similar factual questions as those considered by the jury, may arrive at separate or even different conclusions. That overlap does not negate the fact that the two processes are separate. A trial judge may take into account a jury's verdict but that is just one of many other considerations, see *Mayer*, supra at paras. 42-46.

[75] In this case, I have difficulty accepting the defence's submission that the jury accepted the defence's arguments and did not find Mr. Dimopoulos' credible. That might have been the case if the jury had denied Mr. Dimopoulos any damages. The defence asked the jury to award nothing to Mr. Dimopoulos. While the jury did not accept the figures recommended by the plaintiff, the award of \$37,000 was not insignificant.

[76] Similarly, the award of \$30,000 for future chiropractic care could be interpreted in a number of ways. Once again, the defence invited the jury to deny the award completely. The plaintiff asked for an award that was double what was actually awarded. It is of no benefit to speculate whether the figure represented one treatment per week for a number of years, or two treatments per week for a lesser number of years. What it does suggest is that the jury found both Dr. Lavis and Mr. Dimopoulos credible.

CONCLUDING REMARKS

[77] Based on the evidence before me, I am satisfied that Mr. Dimopoulos met the burden of proving on a balance of probabilities that he sustained a permanent serious impairment of an important physical function.

[78] The defendant's motion is dismissed.

[79] Judgment shall be entered in accordance with the answers given by the jury, namely:

1. General damages - \$37,000
2. Future Chiropractic Care - \$30,000

[80] If the parties are unable to agree on the costs of this action, including the defendant's threshold motion, they may make written submissions not to exceed three pages double-spaced, in addition to a Bill of Costs. The plaintiff will have until January 29, 2016 to make his submissions. The defendant shall have until February 15, 2016 to respond.

Tzimas, J.

Released: January 19, 2016

CITATION: Dimopoulos v. Imad Tafaso Mustafa, 2016 ONSC 429
COURT FILE NO.: CV-05-10263-00
DATE: 2016-01-19

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

PAUL DIMOPOULOS

- and -

IMAD TAFASO MUSTAFA and NIZAR
SULEIMAN SUNCOR ENERGY
PRODUCTS INC. and CERTAS DIRECT
INSURANCE COMPANY and LOWELL
MACKAY

REASONS FOR JUDGMENT

Tzimas, J.

Released: January 19, 2016